

RING THE VET FIRST — speakerphone in your chest pocket, video call if possible. Work together.

PREPARE

Washed arm · long glove · **lots of lubricant, re-lubed at every entry** · cast her on her side (for a leg-back: missing-leg side uppermost; to repel a head: standing, or rear raised on a bale) · hand in a cone, enter slowly, **work between contractions, pause when she strains.**

IDENTIFY BEFORE PULLING ANYTHING

Front leg: fetlock + knee bend the **same** way. Hind leg: they bend **opposite** ways. Tail near hooves = hind end. Trace every leg to its shoulder/hip — with twins, prove both legs are the **same lamb's**.

LEG BACK (ONE FRONT LEG)

1. Push the lamb gently back between strains.
2. Follow shoulder → knee → cannon → fetlock.
3. **Cup the hoof in your palm** (shields the uterus — the detail that matters most).
4. Flex the knee; bring the cupped hoof up over the pelvic brim.
5. Deliver with the traction rules below.

HEAD OUT, NO LEGS (“HUNG LAMB”)

1. Keep the head moist; membranes off the nose.
2. **Repel the head back inside** between strains — feels wrong, is right.
3. Fetch each leg as above (cup every hoof).
4. Snare **behind the ears + through the mouth** holds the head if it slips (never around the jaw alone).
5. Can't repel it in a few minutes → load & drive.

HIND LEGS FIRST (SOLES UP, NO NOSE)

Don't convert — **deliver promptly**: both legs, steady pull down toward her hocks with strains, **brisk through the ribcage** (cord pinches once hips engage). Clear the airway instantly; expect to resuscitate.

HEAD BACK · BREECH · TWINS TANGLED

Phone/video-guided only. Head back: repel, cup the muzzle (never eye sockets), bring it round, snare. Breech: repel rump, cup each hock's hoof, flex up over the brim, deliver hind-first. Twins: repel the less-advanced lamb, deliver the engaged one.

Two failed attempts = load and drive.

AFTER DELIVERY

Airway → rub hard → straw-tickle nostril. Cord intact 1–2 min.

Check for another lamb. Iodine navel; colostrum early. Tell the vet what you did (antibiotic cover is their call).

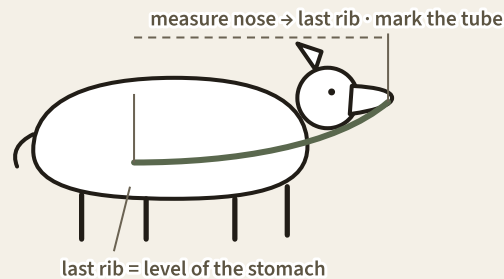
TRACTION RULES: only during her strains · only two legs + head in place (or two hind legs) · downward arc toward her hocks — “down and around, never straight out” · 10–30 seconds of steady arms · never ropes to objects.

STOP = LOAD & DRIVE: two well-applied strains move nothing · you can't identify what you feel · canal dry/swollen beyond lube · her pain escalating. A bedded trailer toward the vet is treatment, not defeat.

Stomach-Tubing a Lamb

FAR FROM HELP · CARD B
OMAFRA · OSU · CORNELL

USE WHEN: lamb $\geq 37^\circ\text{C}$, can hold its head up and swallow, but won't/can't suck. **NEVER TUBE:** a lamb that can't hold its head up or swallow · a lamb $< 37^\circ\text{C}$ older than ~5 h — that lamb gets the IP injection FIRST (Card C): “never stomach tube before an IP glucose injection — milk could kill the lamb” (SRUC).



Measure first. Nose → last rib; tape-mark the tube. It goes in further than feels right — most of the tube.

1. **Measure and mark:** nose → last rib.
2. Sit; lamb upright on your lap, head natural and nose-forward (not stretched up).
3. Wet the tip with colostrum. In at the **side of the mouth**, over the tongue, aiming slightly to the **lamb's left** (food-pipe lies left of and behind the windpipe).
4. Advance slowly. **RIGHT = slides easily**; the lamb may chew and swallow it; with a finger on the throat you feel **two tubes** (windpipe + yours). **WRONG = resistance, coughing, struggling → withdraw completely, start again.**
5. Double-check: ear to the tube's open end — **airflow or crying = windpipe, out.** Silence or stomach gurgle = correct.
6. Syringe on, **plunger out** — warm colostrum by gravity (or very gentle push): **50 ml per kg over ~5 minutes** (~200 ml for a 4 kg lamb). Never fast.
7. **Crimp the tube firmly, then withdraw** in one smooth motion (no drips into the airway).
8. Warm dry pen · recheck 30–60 min · repeat feeds toward ~200 ml/kg by 24 h if still not sucking.

ABORT IF: any cough or gag at any point (withdraw, restart) · you cannot pass it to the mark after two calm attempts (ring the vet — video call, they can see your angle).

IP Glucose Injection — Cold-Lamb Rescue

FAR FROM HELP · CARD C
FAS SCOTLAND · SRUC · EALES & SMALL 1982

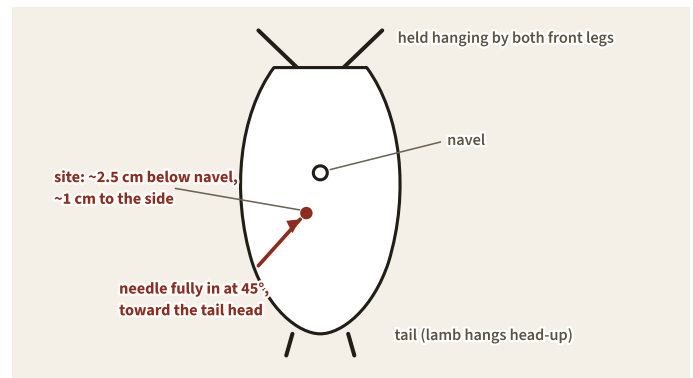
USE ONLY WHEN ALL THREE ARE TRUE: lamb **below 37 °C** · **older than ~5 hours** · **cannot swallow / hold its head up**. A lamb that can swallow gets warmed + tubed instead (Card B). **Do not warm this lamb first** — warming without energy kills it.

PREPARE

Warmed 20% glucose/dextrose, 10 ml per kg — a 4–5 kg lamb takes the full ~45–50 ml. From the kit, or mix: **20 g glucose powder in hot pre-boiled water made up to 50 ml (40%), then dilute half-and-half with warm pre-boiled water** → ~20%, warm-on-the-wrist (never cold). 50–60 ml syringe · **new sterile 1" (25 mm) needle**.

INJECT

1. Ring the vet (speaker, pocket). Draw up the warmed solution.
2. **Hang the lamb by both front legs** (helper ideal; alone: forelegs in one hand, lamb's back against your legs). Gravity drops the organs clear.
3. Site: **~2.5 cm (1") below the navel, ~1 cm (½") to the side**.
4. Full needle in at **45°**, **angled toward the tail head** — two faint pops (skin, wall).
5. **Inject the full dose slowly and steadily**. Withdraw, rub, iodine the site.



THEN — IN THIS ORDER

1. **NOW warm:** warm-air box ~35–40 °C; check every 20–30 min; recheck lamb temp at 30 min. Expect it standing in 30–60 min.
2. Once **≥37 °C and swallowing:** tube 50 ml/kg colostrum (Card B).
3. Re-mother in a pen; confirm sucking.

DO NOT: inject a lamb that can swallow (tube it) · use cold solution · reuse needles between lambs · inject at or forward of the navel itself. Not clearly improving 30–60 min after warming → ring again / drive.

Replacing a Bearing + Retainer

FAR FROM HELP · CARD D
FARMERS WEEKLY · NORTHLAND VETS NZ · AG VICTORIA

RING THE VET FIRST — they may say “bring her in”, or talk you through this while you work.

CHECK FIRST — DO NOT REPLACE IF: the tissue is **torn, black or cold** (a vet must judge if it's salvageable — ring/drive) · she is **actually lambing** behind it (water bag, feet or tail palpable through the cervix) — that is a lambing emergency now, ring immediately.

1. Two people if you can: one holds her standing against a rail. **Raise her hindquarters** (bale under the back legs, or rear held up wheelbarrow-style) — it makes everything easier.
2. **Clean:** plenty of clean warm water with mild antiseptic — dilute iodine to a “**weak-tea colour**” (Northland Vets). All dirt and bedding off. No scrubbing.
3. **Lubricate** the whole surface generously.
4. **Replace:** first **lift the mass up toward her tail** — this un-kinks the bladder; she will often pass urine and the ball shrinks. Then, with the **flat of a clean hand or a closed fist — never fingertips** — slow, steady, patient pressure. **Hold your ground when she strains; gain between strains.** It goes back gradually, then all at once — follow it in so the tip is fully un-inverted.
5. **Retain:** bearing spoon/T-piece — tongue inside the vagina, wings **tied firmly to the wool of her flanks** with the tapes — or a prolapse harness fitted per its instructions.
6. **Aftercare:** ring the clinic and report — pain relief and usually antibiotic cover are warranted (vet-supplied in NZ; ask about pre-supplied cover for your distance *before* lambing). Check the retainer **twice daily** for slip and rubbing.
7. **At lambing:** she can lamb past a spoon (it pushes out) but watch her closely; a harness must be released. **Mark her — bearings recur. Do not breed her again.**

WHILE WAITING / DRIVING: pen her quietly · keep the tissue clean and moist — clean towel soaked in clean lukewarm water held over it · keep other stock and flies off · never let her run or be chased.